

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

December 11, 2025

MEMORANDUM FOR:

(b)(6), (b)(7)(C), (b)(7)(F) DHSc, FACHE
Assistant Director
ICE Health Service Corps

(b)(6), (b)(7)(C), (b)(7)(F)

THROUGH:

CAPT (b)(6), (b)(7)(C), (b)(7)(F), MD, MPH, CCHP, FAAFP
Chief Medical Officer
ICE Health Service Corps

(b)(6), (b)(7)(C), (b)(7)(F)

(b)(6), (b)(7)(C), (b)(7)(F) MS
Chief of Staff
ICE Health Service Corps

CDR (b)(6), (b)(7)(C), (b)(7)(F), RN
Chief, Investigations Unit
ICE Health Service Corps

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FROM:

CDR (b)(6), (b)(7)(C), (b)(7)(F), PsyD
Investigator
ICE Health Service Corps

(b)(6), (b)(7)(C), (b)(7)(F)

SUBJECT:

Mortality Review – Report of Findings
Santos Reyes BANEGAS, (b)(6), (b)(7)(C)

Executive Summary:

On September 18, 2025, the U.S. Immigration and Customs Enforcement (ICE) Health Service Corps received notification of the death of alien Santos Reyes BANEGAS, (b)(6), (b)(7)(C). ICE detained Mr. BANEGAS, a 42-year-old Honduran male, at Nassau County Correctional Center (NCCC), in East Meadow, New York (NY), from September 17 to 18, 2025, the date of his death. Mr. BANEGAS's autopsy and death certificate are pending.

Prior to his detention, on August 27, 2004, ICE Enforcement and Removal Operations (ERO) New York City arrested Mr. BANEGAS and issued a Notice to Appear, Form I-862, charging

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him with violating Section 212(a)(6)(A)(i), as an alien present without admission or parole and held him in ICE custody. On October 16, 2004, ICE verified Mr. BANEGAS' voluntary departure from the United States to Honduras via John F. Kennedy International Airport, New York City, NY. In August 2005, Mr. BANEGAS entered the United States illegally, near Sasabe, Arizona, and ERO Phoenix returned Mr. BANEGAS to Honduras. On September 3, 2019, the Kentucky State Police, in Morehead, Kentucky (KY), arrested Mr. BANEGAS for failure to appear on an unknown charge, and booked him into the Montgomery County Detention Center, in Mt. Sterling, KY. On October 24, 2019, ERO removed Mr. BANEGAS to Honduras. On September 17, 2025, ERO New York City, Long Island Fugitive Operations, ICE Homeland Security Investigations, and U.S. Customs and Border Protection arrested Mr. BANEGAS for unlawful entry into the United States without admission or inspection. ERO issued Mr. BANEGAS a Notice of Intent/Decision to Reinstate Prior Order of Removal, form I-871, and an I-205 Warrant of Removal/Deportation. ERO detained Mr. BANEGAS at NCCC.

On September 17, 2025, at 11:22 a.m., during his intake screening, Mr. BANEGAS reported a medical history of hypertension [(HTN); elevated blood pressure], daily alcohol use, and a motor vehicle accident, resulting in a lower extremity fracture and altered gait. A registered nurse (RN) noted elevated BP and heart rate, positive symptoms of alcohol withdrawal, a normal respiration rate and body temperature, and mild abdominal tenderness. Later in the day, a medical doctor (MD) evaluated Mr. BANEGAS, documented a current umbilical hernia, mild abdominal pain, tremors, and alcohol withdrawal symptoms. The MD started the Clinical Institute Withdrawal Assessment (CIWA) for Alcohol Protocol, and prescribed vitamins and medicine to treat pain and alcohol withdrawal.

On September 18, 2025, at 6:25 a.m., custody staff observed Mr. BANEGAS unresponsive in his cell, announced a medical emergency, initiated cardiopulmonary resuscitation (CPR), and applied an automated external defibrillator (AED). Between 6:26 and 6:44 a.m., medical staff assumed CPR and administered oxygen and intravenous saline. At 6:45 a.m., emergency medical services (EMS) personnel arrived at the scene and pronounced Mr. BANEGAS deceased.

Mortality Finding:

As a result of Mr. BANEGAS' death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures or practices are warranted; and identify issues requiring further study.

On December 11, 2025, Commander (CDR) (b)(6),(b)(7)(C),(b)(7)(F) psychologist, investigator, IHSC Investigations Unit, discussed the events surrounding Mr. BANEGAS' death with IHSC Chief Medical Officer Captain (CAPT) (b)(6),(b)(7)(C),(b)(7)(F) medical doctor, physician reviewer. The reviewers determined NCCC did not provide Mr. BANEGAS medical care within the safe limits of practice. Specifically, the reviewers determined, 1) NCCC medical staff did not

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appropriately monitor or treat Mr. BANEGAS' alcohol withdrawal symptoms. 2) NCCC did not coordinate care between disciplines. 3) NCCC medical records did not contain vital signs and assessments reportedly completed by health staff. 4) NCCC custody staff did not look inside cells during rounds/wellness checks to ensure proof of life.

Due to factors outside of IHSC's control, the IHSC investigator was unable to obtain all relevant policies and procedures, interview staff, or conduct an on-site review. The following summary of events was based on: 1) Mr. BANEGAS' NCCC medical records; 2) incident and notification reports; 3) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 4) Mr. BANEGAS' NCCC detention file; 5) video footage from September 17 to 18, 2025; 6) information reported by the ICE Office of Professional Responsibility (OPR) External Reviews and Analysis Unit (ERAU), based on staff interviews OPR ERAU conducted at NCCC; 7) applicable NCCC policies and procedures; and 8) applicable accreditation standards.

Summary of Events

On September 17, 2025, at 11:22 a.m., an RN completed Mr. BANEGAS' intake screening, using a Spanish interpreter, and documented his reported histories of HTN without medication, daily alcohol use, and a motor vehicle accident six months prior, resulting in a lower extremity fracture and altered gait. The RN noted a normal respiration rate and body temperature, an elevated blood pressure of 135/92 millimeters of mercury [(mmHg); normal less than 120/80 mmHg], an elevated heart rate at 120 beats per minute [(bpm); normal range 60 – 100 bpm], positive symptoms of alcohol withdrawal, mild abdominal tenderness, and his denial of mental health history.

- On the same date, at 1:32 p.m., an MD evaluated Mr. BANEGAS, documented his medical histories, his current umbilical hernia, mild abdominal pain, tremors, and alcohol withdrawal symptoms, started the CIWA protocol (assesses severity of alcohol withdrawal symptoms), and prescribed the following medications: acetaminophen (treats pain) 325 milligrams (mg), two tablets, by mouth (PO), twice a day (BID), for six days; diazepam (benzodiazepine; reduces complications of alcohol withdrawal) 10 mg, one tablet, PO, immediately (day 1); diazepam 5 mg, two tablets, PO, BID, on days 2 – 3; diazepam 5 mg, one tablet, PO, BID, on days 4 – 5; and diazepam 5 mg, one tablet, PO, at bedtime, on day 6. The MD ordered Mr. BANEGAS a low bunk pass for seizure precautions for seven days, drug and alcohol assessments and vital signs, BID, for seven days, and prescribed thiamine (vitamin B1), folic acid (vitamin B9), and a multivitamin, one tablet each, in the morning, for 30 days.

On September 18, 2025, at 6:25 a.m., during morning count, a custody officer observed Mr. BANEGAS unresponsive in his cell, activated a body alarm, announced a medical emergency, and immediately initiated CPR until medical staff arrived.

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- At 6:26 a.m., custody staff applied an AED on Mr. BANEGAS' chest. Medical staff arrived at the housing unit, assessed Mr. BANEGAS, noted his unresponsiveness, lack of pulse, and cool, pale skin, assumed CPR, and administered oxygen. The AED did not advise any shocks.
- At 6:30 a.m., custody staff called 911 for emergency response.
- At 6:33 a.m., medical staff inserted an intravenous catheter into Mr. BANEGAS' right forearm, administered normal saline, and continued CPR until EMS personnel arrived.
- At 6:45 a.m., EMS personnel arrived at the scene and pronounced Mr. BANEGAS deceased.

The reviewers used the following accreditation standards for this review: U.S. Department of Justice (DOJ) United States Marshal Service (USMS) Federal Performance-Based Detention Standards (FPBDS). The MRC identified the following program strengths, weaknesses, and recommendations during this review.

Strengths

The reviewers identified the following strengths: 1) NCCC custody staff promptly started CPR and obtained an AED within one minute of discovering Mr. BANEGAS. 2) NCCC medical staff responded to the scene one minute after custody called the medical emergency. 3) NCCC custody staff promptly called 911 allowing for a quick response from EMS.

Weaknesses

1. Inadequate Medical Care.

- On September 17, 2025, the MD documented intent to initiate CIWA protocol; however, the MD did not document Mr. BANEGAS' CIWA score at the time of the assessment, and the NCCC medical staff did not monitor Mr. BANEGAS or conduct a subsequent CIWA.

Applicable standards of care for this finding:

- USMS FPBDS 2025, Section B, *Health Care*.
- NCCC Local Policy, Inmate Admission Process 2019, Section I, *Medical Unit*.

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2. Inadequate Continuity of Care.

- On September 17, 2025, Mr. BANEGAS arrived at the facility with positive symptoms of alcohol withdrawal. An RN did not document a referral to higher level of medical or behavioral health care.
- NCCC medical staff did not assess vital signs or otherwise monitor withdrawal symptoms after his initial intake screening.
- NCCC medical staff did not alert custody staff of Mr. BANEGAS' alcohol withdrawal symptoms prior to clearing him for placement in general population.

Applicable standards of care for this finding:

- USMS FPBDS 2025, Section B, *Health Care*
- NCCC Local Policy, Inmate Admission Process 2019, Section I, *Medical Unit*.

3. Documentation

- Medical staff reported they obtained Mr. BANEGAS' vital signs during the evening of September 17, 2025; however, there are no vital signs documented, after his initial intake, in Mr. BANEGAS' medical record.

Applicable standards of care for this finding:

- USMS FPBDS 2025, Section B, *Health Care*.
- USMS FPBDS 2025, Section A, *Staff Training*.

4. Staff Training.

- NCCC custody staff did not ensure inmate wellness during overnight rounds and cell checks.

Applicable standards of care for this finding:

- USMS FPBDS 2025, Section A, *Staff Training*.
- NCCC Local Policy, Post Orders 1992, Section IV, *Supervision of Inmates in the Housing Area*.

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Recommendations

- Forward these findings to the IHSC Deputy Assistant Director (DAD) of Health Care Compliance Division (HCCD).
- The IHSC DAD of HCCD will share these findings through appropriate communication channels to ICE, and NCCC's facility administrator and health authority, for review and any action(s) deemed appropriate.
- The respective IHSC HCCD unit and ICE will ensure a corrective action plan, if applicable, is implemented and sustained.

End of Report.